

ST. MARY GENERAL HOSPITAL — INTERNAL MEDICINE

ADMISSION NOTE

Name: Robert Hale Age/Sex: 64 / Male MRN: 884213

Date of Admission: 12/05/2026 Ward: 4B, Bed 12

Attending: Dr. A. Mensah

PRESENTING COMPLAINT

Productive cough with yellow sputum for 4 days, fever, and breathlessness on exertion for 2 days. One episode of rigors last night.

HISTORY OF PRESENT ILLNESS

64-year-old male, ex-smoker (quit 5 years ago, 30 pack-years), presents with a 4-day history of productive cough, subjective fever, and progressive dyspnoea. No chest pain. No haemoptysis. No recent travel. No known sick contacts.

PAST MEDICAL HISTORY

- Hypertension (since 2014)
- Type 2 diabetes mellitus (since 2018)
- No prior hospital admissions.

ALLERGIES

Penicillin — causes rash (documented by patient).

MEDICATIONS ON ADMISSION (pre-admission regimen)

Drug	Dose	Route	Frequency
Amlodipine	5 mg	PO	Once daily
Metformin	500 mg	PO	Twice daily
Atorvastatin	20 mg	PO	Once at night

EXAMINATION

Temp 38.7°C, HR 104/min, BP 138/86 mmHg, RR 24/min, SpO2 91% on room air. Chest: crackles at right base. Heart sounds normal. Abdomen soft.

PROVISIONAL DIAGNOSIS

Community-acquired pneumonia (right lower lobe). CURB-65 score 2.

PLAN

Admit. Start IV ceftriaxone (penicillin allergy noted — cephalosporin chosen after discussion). Send CXR, FBC, U&E, CRP, blood cultures. Continue home metformin and atorvastatin. Oxygen to keep SpO2 > 94%.

PROGRESS NOTE — Day 2 (13/05/2026)

Reviewed on morning round. Patient reports cough is unchanged, fever settling. Overnight nursing notes mention one witnessed episode of coughing during drinking, with mild choking.

ASSESSMENT

CXR reviewed: right lower lobe consolidation. Given the witnessed episode of choking on fluids and the dependent-lobe distribution, the picture is most consistent with **aspiration pneumonia** rather than typical community-acquired pneumonia. Speech & language therapy referral made for swallow assessment.

INVESTIGATIONS

FBC: WCC 15.2 (high), Hb 13.4, Plt 280.

CRP: 142 mg/L.

U&E: Na 137, K 4.1, Urea 7.2, Creatinine 98 µmol/L.

Blood cultures: **sent 12/05/2026 — report awaited.**

Random glucose: 11.8 mmol/L.

PLAN

Continue IV ceftriaxone. Add metronidazole given aspiration concern. Chase blood cultures. Await SALT swallow assessment. Encourage oral intake as tolerated.

PROGRESS NOTE — Day 4 (15/05/2026)

Patient clinically improved. Afebrile for 48 hours. SpO2 96% on room air. Crackles reduced. CRP down to 48 mg/L. Tolerating soft diet; SALT cleared for normal consistency with no aspiration on bedside assessment.

INVESTIGATIONS

Repeat CRP: 48 mg/L (improving).

Blood cultures (taken 12/05/2026): **still no growth reported — final report awaited at time of writing.**

Repeat glucose profile: fasting 8.2 mmol/L.

PLAN

Switch IV antibiotics to oral co-amoxiclav to complete course. Plan discharge tomorrow if remains stable.

Diabetes team to advise on glucose control.

DISCHARGE SUMMARY SHEET

Name: Robert Hale Age/Sex: 64 / Male MRN: 884213

Date of Admission: 12/05/2026 Date of Discharge: 16/05/2026

FINAL DIAGNOSIS

Community-acquired pneumonia, right lower lobe. Type 2 diabetes mellitus (suboptimal control during admission).

COURSE IN HOSPITAL

Admitted with right lower lobe pneumonia and treated with IV antibiotics. Brief concern regarding possible aspiration prompted a swallow assessment, which was normal. Clinically improved, switched to oral antibiotics and discharged in stable condition.

PROCEDURES

None. (No invasive procedures performed.)

DISCHARGE MEDICATIONS

Drug	Dose	Route	Frequency
Co-amoxiclav	625 mg	PO	Three times daily for 5 days
Metformin	500 mg	PO	Twice daily
Atorvastatin	20 mg	PO	Once at night

ALLERGIES

Penicillin (rash).

FOLLOW-UP

- GP review in 1 week.
- Chest clinic in 6 weeks with repeat chest X-ray to confirm resolution.
- Diabetes nurse to review glucose control.

PENDING RESULTS AT DISCHARGE

Blood culture (taken 12/05/2026) — **final result still awaited**. GP to follow up and adjust antibiotics if positive.

CONDITION AT DISCHARGE

Stable. Afebrile, SpO2 96% on room air, mobilising independently.